

Patient Name: [REDACTED]
MRN: [REDACTED]
DOB: [REDACTED] Sex: Female
Account Number: [REDACTED]
Visit Date
Discharge date
Patient Type: Bedded
Location:
Attending Physician:

ICB-0-3

Carcinoma, papillary 8260/3
Site: thyroid, NOS C73.9 2/14/11
[Signature]

Final Pathology Diagnosis Report

Accession Number: Senior Pathologist:

Collected Date/Time:

Received Date/Time:

Verified Date/Time:

Final Pathologic Diagnosis

A) CENTRAL NECK DISSECTION, RIGHT (DISSECTION):

- Metastatic papillary thyroid carcinoma in one of four lymph nodes (1/4).
- Benign thymus gland with involutional changes.

B) THYROID AND LEFT CENTRAL NECK DISSECTION (THYROIDECTOMY AND NECK DISSECTION):

- Papillary thyroid carcinoma (See synoptic report).
- Metastatic papillary thyroid carcinoma in 6 of 7 lymph nodes, with focal extra capsular extension (6/7).

C) NECK DISSECTION, LEFT, LEVEL 2, 3, 4 (DISSECTION):

- Metastatic papillary thyroid carcinoma in one of three lymph nodes (1/3).

Note: The case was discussed in the intradepartmental consensus conference on [REDACTED] at [REDACTED] and reviewed in conjunction with one or more attending pathologists in this department who concur(s) with the above diagnosis.

I attest I have personally reviewed the specimen/slides and agree with the above findings.

Electronic Signature

Verified:

Resident: ?

UUID: FB43CFF1-85FF-46D6-BCE2-221B2854A390
TCGA-DO-A1K0-01A-PR Redacted



Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
Pathologic Discrepancy		X
or Malignancy History		X
al/Synchronous Primary noted		X
is (circle):	QUALIFIED	DISQUALIFIED
never final		
Date reviewed:	2/16/11	

Synoptic Report

A: THYROID, RESECTION
PROCEDURE:

Other (specify): Total thyroidectomy with central neck dissection and left neck dissection (levels 2,3,4)

Printed:

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RECEIVED:

Fresh

SPECIMEN INTEGRITY:

Intact

SPECIMEN SIZE:

Right lobe: 5.1 x 2.4 x 0.5 cm

Left lobe: 4.2 x 4.1 x 2.7 cm

Isthmus / pyramidal lobe: 2 x 0.5 x 0.3 cm

Central compartment: 3 x 1.2 x 0.6 cm

Left neck dissection: 10 x 3.5 x 2 cm

SPECIMEN WEIGHT:

Specify: 46 g

TUMOR FOCALITY:

Multifocal (specify)

Bilateral

DOMINANT TUMOR:

TUMOR LATERALITY

Left lobe

TUMOR SIZE

Greatest dimension: 4.2 cm

Additional dimensions: 4.1 x 2.7 cm

HISTOLOGIC TYPE

Papillary carcinoma

Variant, specify:

Classical

ARCHITECTURE:

Classical (papillary)

CYTOMORPHOLOGY

Classical

MARGINS

Margins uninvolved by carcinoma

Distance of invasive carcinoma to closest margin: 1 mm

LYMPH-VASCULAR INVASION

Not identified

PERINEURAL INVASION

Not identified

EXTRATHYROIDAL EXTENSION

Present

Minimal

Printed:

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SECONDARY TUMOR:

TUMOR LATERALITY

Right lobe

TUMOR SIZE

Greatest dimension: 0.1 cm

HISTOLOGIC TYPE

Papillary carcinoma ✓

Microcarcinoma (occult, latent, small, papillary microtumor)

ARCHITECTURE:

Follicular

CYTOMORPHOLOGY

Classical

MARGINS

Margins uninvolved by carcinoma

Distance of invasive carcinoma to closest margin: 3 mm

LYMPH-VASCULAR INVASION

Not identified

EXTRATHYROIDAL EXTENSION

Not identified

PATHOLOGIC STAGING (pTNM):

pT3: Tumor more than 4 cm, limited to thyroid or any tumor with minimal extrathyroidal extension (eg. extension to sternothyroid muscle or perithyroid soft tissues)

REGIONAL LYMPH NODES (pN)

pN1b: Metastases to unilateral, bilateral or contralateral cervical (Levels I, II<III< IV< V) or retropharyngeal or superior mediastinal lymph nodes (Level VII)

Number examined: 14

Number involved: 9

LYMPH NODE, EXTRANODAL EXTENSION

Present

DISTANT METASTASIS (pM)

Not applicable

***ADDITIONAL PATHOLOGIC FINDINGS:**

None identified

Clinical History

yr. old white female with 1 yr. history of left sided enlarging thyroid mass. ✓

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Pre/Post-operative Diagnosis

Papillary thyroid cancer.

Gross Anatomic Description

(Dictated by Dr. on

Reviewed by Dr.)

Specimen received in three containers.

Specimen A: Designated "right central neck dissection" is received in formalin labeled with the patient's name, MRN and "right central neck dissection". Specimen consists of multiple pieces of fibrofatty tissue measuring an aggregate of 3.5 x 2 x 1.5 cm. Palpating/sectioning the tissue reveals several small lymph nodes and probable thymus tissue. 100% of the specimen is submitted.

Section code: A1 – fat; A2 – three possible lymph nodes; A3 – A4 – probable thymus, bisected.

Specimen B: Designated "total thyroid, left neck central neck dissection" is received fresh for Tumor Bank labeled with the patient's name and "total thyroid, left neck central neck dissection". Specimen consists of one total thyroid with attached central neck dissection weighing 46 gm. The right lobe measures 5.1 x 2.4 x 0.5 cm and the left lobe measures 5.1 x 3.9 x 3.6 cm. The isthmus measures 2 x 0.5 x 0.3 cm. The central neck dissection measures 3 x 1.2 x 0.6 cm. Serial sections of the right hemi-thyroid reveal a homogenous unremarkable parenchyma without distinct lesions or nodules. Serial sections of the left hemi-thyroid reveal a large mass occupying the majority of the lobe which has a soft, gelatinous, yellow appearance which is somewhat gritty on cut section and has areas of fibrosis. No necrosis, hemorrhage or thick encapsulation is identified. The mass measures in greatest dimension of 4.2 x 4.1 x 2.7 cm. The mass appears to extend out of the thyroid. Palpating the central neck dissection reveals multiple firm lymph nodes. A portion of the tumor is given to Tumor Bank.

Inking code: Green - right side of thyroid; Blue - left side of thyroid.

Section code: B1 – two possible lymph nodes; B2 – one lymph node, bisected; B3 – one possible lymph node, bisected; B4-B8 – representative sections of right hemi-thyroid from more superior aspect to more inferior aspect; B9-B12 – representative section of left hemi-thyroid with mass and outer capsule; B13 – representative section of mass (no margin); B14 – thyroid tissue, just inferior to mass; B15 – one lymph node bisected.

Specimen C: Designated "left neck dissection, level 2, 3, 4" is received in formalin labeled with the patient's name and "left neck dissection". Specimen consists of one un-oriented lateral neck dissection with attached internal jugular vein. The vein measures 4.5 x 0.5 x 0.4 cm. The attached fibrofatty tissue measures up to 10 x up to 3.5 x up to 2 cm. At one end (which is not designated) is a lymph node which is hanging loosely from adjoining tissue. Serial sections of the largest nodal appearing tissue (noted in the upper third of the specimen) reveals a cystic lesion filled with red/brown fluid. A representative photograph is taken.

Section code: C1-C2 – representative sections of largest nodal tissue (cystic in nature); C3 – one possible lymph node from midportion of specimen (bisected); C4 – lymph node which is loosely hanging from opposite end of neck dissection (bisected);

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C5 – representative sections of vein.

Printed:

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